- Initial history and physical
- All clinical notes including rationale for proposed treatment plan
- All ovarian stimulation sheets for timed intercourse, IUI, and/or IVF cycles
- All embryology reports
- All operative reports
- Laboratory report FSH, AMH, estradiol, and any other pertinent information
- Ultrasound report antral follicle count and any other pertinent information
- HSG report
- Semen analysis

Policy Guidelines

According to the **American Society of Reproductive Medicine** (ASRM) and **American Society for Clinical Oncology** (ASCO) medical practices and guidelines fertility preservation services are defined as those procedures indicated for an individual facing infertility due to chemotherapy, pelvic radiotherapy, or other surgical procedures expected to render one permanently infertile (e.g., hysterectomy, oophorectomy)

AMERICAN SOCIETY OF CLINICAL ONCOLOGY (ASCO): According to the ASCO guidelines on fertility preservation in patients with cancer, oocyte and embryo cryopreservation are considered standard practice (Oktay et al., 2018). ASCO purports that the field of ovarian tissue cryopreservation (OTC) is advancing quickly and may evolve to become standard therapy in the future.

AMERICAN SOCIETY FOR REPRODUCTIVE MEDICINE (ASRM): An ASRM committee issued an opinion that OTC and reimplantation of tissue is experimental, although it may be an option for individuals in which gonadotoxic treatment is necessary immediately or among prepubertal girls (Hayes Report: Health Technology, 2021).

AMERICAN SOCIETY FOR REPRODUCTIVE MEDICINE (ASRM): The value of PGT-A as a routine screening test for all patients undergoing in vitro fertilization has not been demonstrated. Although some earlier single-center studies reported higher live birth rates after PGT-A in favorable-prognosis patients, recent multicenter, randomized control trials in women with available blastocysts concluded that the overall pregnancy outcomes via frozen embryo transfer were similar between PGT-A and conventional in vitro fertilization. The value of PGT-A to lower the risk of clinical miscarriage is also unclear, although these studies have important limitations.